

CPT1 Common-Illness Summary · สารบัญ

32 กลุ่มอาการ (32 symptom groups) common-illness decision pages: SCHOLAR-MAC → red flags → illness-script patterns → medication differential. Companion to the drug-class reference PDFs.

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CPT1 common-illness — shared red flags & patient context

This is the material every one of the 32 symptom pages (`cpt-symptom-*`) points back to instead of repeating. Source: pages 17–18 of the deck. Each symptom page carries only its own **symptom-specific** red flags in its own "RED FLAGS / REFER" section.

⚠️ NAMING INCONSISTENCY IN THE DECK ITSELF

Page 15 (history-taking framework) calls it **SCHOLAR-MACO**. All 32 symptom pages (pages 19–50) say **SCHOLAR-MAC focus**. Same framework — the deck drops the **O** (Other risks) on every symptom page. This is the deck's own inconsistency, not an extraction error; the symptom pages use their own wording ("SCHOLAR-MAC").

SCHOLAR — analyzing the symptom: **S**ymptoms · **C**haracteristics (quality/severity) · **H**istory/onset (timing and context) · **O**nset/other associated symptoms · **L**ocation · **A**ggravating/alleviating factors · **R**emitting/previous treatment.

MACO — analyzing the patient and medicines: **M**edications (current, OTC, herbal, vaccines) · **A**llergies/ADRs · **C**omorbidities/conditions (pregnancy, lactation, renal, hepatic) · **O**ther risks (age, immune status, occupation, exposure, ability to use the medicine).

REFERRAL RED-FLAG TABLE (PAGE 17)

Aimed specifically at the referral decision — the *clinical concern* is the feared diagnosis behind each red flag, not a diagnosis itself.

RED FLAG SYMPTOM	CLINICAL CONCERN (POSSIBLE SERIOUS DISEASES)	RECOMMENDED ACTION
Fever $\geq 39^{\circ}\text{C}$ or fever > 3 days	Bacterial infection, pneumonia, sepsis, influenza complications, or COVID-19 complications	Refer for physician evaluation
Cough > 3 weeks	Tuberculosis, chronic bronchitis, lung cancer, post-infectious cough, or asthma	Refer for physician evaluation
Sore throat > 7 days	Streptococcal pharyngitis, peritonsillar abscess, infectious mononucleosis, or head and neck malignancy	Refer for physician evaluation
Purulent nasal discharge > 10 days	Acute bacterial rhinosinusitis, possible sinus complications	Refer for physician evaluation
Hemoptysis (small amount)	Tuberculosis, lung cancer, bronchiectasis, pulmonary infection, or pulmonary embolism	Refer for physician evaluation
Respiratory distress (severe dyspnea, labored breathing)	Severe pneumonia, asthma exacerbation, acute respiratory failure, pulmonary embolism	Emergency referral to ER
Stridor or drooling	Epiglottitis, airway obstruction, peritonsillar abscess, retropharyngeal abscess	Emergency referral to ER
$\text{SpO}_2 < 96\%$	Hypoxemia due to pneumonia, COVID-19, acute respiratory distress, or lung disease	Emergency referral to ER
Altered consciousness	Sepsis, severe hypoxia, meningitis, encephalopathy	Emergency referral to ER
Severe hemoptysis	Massive pulmonary hemorrhage, tuberculosis, lung cancer, pulmonary embolism	Emergency referral to ER

References (page 17): CDC Emergency warning signs of respiratory illness; NICE Sore throat and respiratory infection guidance; IDSA Guideline for Acute Bacterial Rhinosinusitis; AAO-HNS Adult Sinusitis Clinical Practice Guideline.

GENERAL RED FLAGS (PAGE 16, ภาษาไทย — THE QUESTION THAT COMES BEFORE "จะจ่ายยาอะไร")

สัญญาณทั่วไป (general signals): - อาการรุนแรง/ทรุดเร็ว/หมดสติ/ซึม — severe/rapidly deteriorating/unconscious/drowsy - ไข้สูงมากหรือไข้ในกลุ่มเสี่ยง — very high fever, or fever in a risk group - หอบเหนื่อย, SpO₂ ต่ำ, เจ็บหน้าอก, ชัก — dyspnea, low SpO₂, chest pain, seizure - ปวดรุนแรงที่สุดในชีวิต, เลือดออกมาก, ขาดน้ำ — worst pain of their life, heavy bleeding, dehydration - อายุสุดขั้ว, ตั้งครรภ์, ภูมิคุ้มกันบกพร่อง, โรคร่วมควบคุมไม่ได้ — extremes of age, pregnancy, immunocompromise, uncontrolled comorbidity

สัญญาณเฉพาะอาการ (symptom-specific signals, examples given on the slide): - ปวดศีรษะ + คอแข็ง/อ่อนแรง/พูดไม่ชัด — headache + neck stiffness/weakness/slurred speech - ห้องเสียง + เลือด/ขาดน้ำ/ไข้สูง/ผู้สูงอายุ — diarrhea + blood/dehydration/high fever/elderly - UTI + ไข้, ปวดเอว, ตั้งครรภ์, ผู้ชาย, เด็ก — UTI + fever, flank pain, pregnancy, male, child - ผื่น + mucosal involvement/skin pain/ผื่นลามเร็ว — rash + mucosal involvement/skin pain/rapid spread - ตาแดง + ปวดตา/ตามัว/แสงจ้า/ใส่ contact lens — red eye + eye pain/blurred vision/photophobia/contact lens

References (page 16): CDC Antibiotic Prescribing & Use: Adult/Pediatric outpatient care 2024–2025; NICE CKS guidance; Thai PBS/NHSO news 21 Aug 2024 "32 กลุ่มอาการ"; NHSO.go.th ข่าวร้านยาคุณภาพ 2567–2568.

PATIENT & EXPOSURE CONTEXT (PAGE 18)

"Patient context can change a 'minor' symptom into a high-risk presentation."

- Age extremes, frailty, pregnancy/postpartum, or inability to maintain hydration
- Diabetes, cardiovascular disease, kidney/liver disease, asthma/COPD, seizure disorder
- Immunosuppression, cancer, HIV, transplantation, neutropenia, long-term corticosteroids
- Recent surgery, hospitalization, travel, trauma, procedures, sick contacts, or outbreaks

MEDICINE-RELATED CONTEXT (PAGE 18)

- Prescription, non-prescription, herbal, recreational, and recently *stopped* medicines
- New medicine or dose change temporally related to the symptom
- Allergies and previous serious adverse drug reactions
- Duplicate ingredients, interactions, contraindications, adherence, and self-treatment already tried

References (page 18): Nichol JR, et al. Medical History. StatPearls/NCBI Bookshelf. Ruiz ME. Risks of self-medication practices. Curr Drug Saf. 2010.

HOW THIS CONNECTS TO THE 32 SYMPTOM PAGES

Every cpt-symptom-* page follows the same shape: **SCHOLAR-MAC focus** (4 symptom-specific questions) → its own **RED FLAGS / REFER** (symptom-specific, on top of the general list above) → the same **red-flag gate** (YES → same-day/urgent/emergency referral; NO → identify the dominant illness-script pattern) → **3–5 pattern branches**, each naming a suspected diagnosis → a **medication-related differential** → **references**. This page is what every one of them links to instead of re-printing the tables above.

1. Dizziness / Vertigo

SCHOLAR-MAC FOCUS

- Onset and duration: seconds, hours, or persistent?
- Triggered by head movement, standing up, or spontaneous?
- True spinning vertigo vs lightheadedness?
- Associated hearing loss, tinnitus, nausea, or neurologic symptoms?

RED FLAGS / REFER

Focal neurologic deficit, severe gait instability, new hearing loss, severe headache, chest pain/syncope, or persistent vomiting → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Brief vertigo triggered by position change	Suspected benign paroxysmal positional vertigo (BPPV)
Acute prolonged vertigo after viral illness; no neuro deficit	Suspected vestibular neuritis
Vertigo with tinnitus or fluctuating hearing loss	Suspected Ménière disease / labyrinthitis
Lightheadedness on standing; poor intake or dehydration	Suspected orthostatic hypotension / dehydration

MEDICATION-RELATED DIFFERENTIAL

Recent/new antihypertensives, diuretics, or sedatives → suspected drug-induced dizziness / orthostatic symptoms.

RELATED DRUG CLASSES

- *Antihypertensive Agents* — new antihypertensive → orthostatic dizziness
 - *Diuretics* — new diuretic → orthostatic dizziness
- ⚠ "Sedatives" is named but not linked — too many candidate classes (antihistamines, benzodiazepines, opioids) with no single unambiguous page; would be guessing which one.

REFERENCES

AAO-HNS BPPV guideline, PMID 28248609 | AAFP Dizziness: Diagnostic Approach

Drug-induced symptom ref: AAFP. Orthostatic Hypotension: A Practical Approach (common causative drugs include antihypertensives, diuretics, psychoactive medicines).

2. Headache

SCHOLAR-MAC FOCUS

- First or recurrent headache? Sudden or gradual onset?
- Location, severity, throbbing vs pressing, and duration?
- Photophobia, nausea, aura, fever, sinus symptoms, or trauma?
- Medication overuse, pregnancy, hypertension, or neurologic symptoms?

RED FLAGS / REFER

Thunderclap headache, fever/neck stiffness, new neurologic deficit, papilledema, immunosuppression, pregnancy-related severe headache, or post-trauma headache → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Bilateral pressing headache; stress; no nausea	Suspected tension-type headache
Unilateral throbbing ± nausea, photophobia, or aura	Suspected migraine
Facial pressure, congestion, purulent discharge	Suspected acute rhinosinusitis-related headache
Daily headache with frequent analgesic use	Suspected medication-overuse headache

MEDICATION-RELATED DIFFERENTIAL

Frequent analgesic or triptan use → suspected medication-overuse headache.

RELATED DRUG CLASSES

- *Drugs Used in Migraine Headache* — triptan overuse + suspected migraine pattern branch

REFERENCES

SNNOOP10 secondary headache red flags, PMID 30587518 | AAFP Acute Headache in Adults

Drug-induced symptom ref: AAFP. Acute Headache in Adults; medication-overuse headache should be considered in recurrent headache.

3. Musculoskeletal Pain

SCHOLAR-MAC FOCUS

- Where is the pain: muscle, joint, neck, or lower back?
- Trauma or overuse? Acute vs chronic?
- Swelling, redness, morning stiffness, weakness, or numbness?
- Functional limitation or radiation of pain?

RED FLAGS / REFER

Major trauma, deformity, hot swollen joint with fever, progressive weakness, saddle anesthesia, urinary retention, or severe unremitting pain → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Localized pain after lifting or exercise	Suspected muscle strain / sprain
Joint pain with overuse but no systemic signs	Suspected tendinitis / overuse injury
Low back pain radiating below knee	Suspected lumbar radiculopathy / sciatica
Chronic knee or hand pain worse with use	Suspected osteoarthritis

MEDICATION-RELATED DIFFERENTIAL

Recent statin use → suspected statin-associated muscle symptoms (myalgia).

RELATED DRUG CLASSES

- *Lipid-lowering Agents* — statin → statin-associated muscle symptoms
- *Drugs in Osteoarthritis & Rheumatoid Arthritis* — suspected osteoarthritis pattern branch

REFERENCES

ACP low-back-pain guideline, PMID 28192789 | ACR/AF osteoarthritis guideline, PMID 31908149

Drug-induced symptom ref: Stroes ES, et al. Statin-associated muscle symptoms. Eur Heart J. 2015. PMID: 25694464.

4. Toothache

SCHOLAR-MAC FOCUS

- Tooth pain with hot/cold sensitivity, chewing pain, or swelling?
- Duration and progression?
- Facial swelling, fever, or difficulty opening the mouth?
- Bleeding gums or foul taste?

RED FLAGS / REFER

Facial swelling, trismus, dysphagia, fever, rapidly worsening pain, or airway concern → same-day dental/medical referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Sharp pain with sweet/cold exposure	Suspected dental caries with pulp irritation
Spontaneous throbbing pain; pain on biting	Suspected pulpitis / apical periodontitis
Tooth pain with gum swelling or pus	Suspected dental abscess
Jaw pain with chewing and clicking	Suspected temporomandibular disorder

MEDICATION-RELATED DIFFERENTIAL

Bisphosphonate or denosumab exposure with jaw/tooth pain → suspected medication-related osteonecrosis of the jaw.

RELATED DRUG CLASSES

- *Drugs Regulating Bone-Mineral Homeostasis* — bisphosphonate/denosumab → medication-related osteonecrosis of the jaw

REFERENCES

ADA urgent dental pain antibiotic guideline, PMID 31668170 | ADA Oral Analgesics for Acute Dental Pain
Drug-induced symptom ref: AAOMS Position Paper on Medication-Related Osteonecrosis of the Jaw.

5. Dysmenorrhea

SCHOLAR-MAC FOCUS

- Cramping pain with menses: since menarche or new onset?
- Timing relative to menstrual flow and severity?
- Heavy bleeding, dyspareunia, infertility, or pelvic pain between periods?
- Possibility of pregnancy?

RED FLAGS / REFER

Positive pregnancy test, syncope, severe sudden pelvic pain, fever, abnormal vaginal bleeding, or severe new-onset pain → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Recurrent crampy lower abdominal pain with menses; normal exam history	Suspected primary dysmenorrhea
Pain with heavy bleeding or enlarged tender uterus history	Suspected adenomyosis / fibroid-related pain
Pain with dyspareunia, infertility, or chronic pelvic pain	Suspected endometriosis
Missed period with pain and spotting	Suspected ectopic pregnancy until proven otherwise

MEDICATION-RELATED DIFFERENTIAL

Recent misoprostol or prostaglandin use → suspected medication-related uterine cramping / pelvic pain.

RELATED DRUG CLASSES

None linked — see note below.

⚠️ No link made anywhere on this page — misoprostol/prostaglandin agents have no dedicated page among the 52 drug-class pages, and the pattern branches (adenomyosis, endometriosis, ectopic pregnancy) don't name a treatment class in the deck. Linking any of the 52 pages here would be guessing.

REFERENCES

ACOG dysmenorrhea/endometriosis in adolescents | AAFP Dysmenorrhea

Drug-induced symptom ref: ACOG. Medication Abortion Up to 70 Days of Gestation; cramping is an expected effect of misoprostol.

6. Abdominal Pain

SCHOLAR-MAC FOCUS

- Where is the pain: epigastric, RUQ, periumbilical, LLQ, or diffuse?
- Acute or recurrent? Colicky or constant?
- Associated nausea, vomiting, diarrhea, urinary or menstrual symptoms?
- Food-related, bowel-related, or movement-related?

RED FLAGS / REFER

Peritonitis, severe localized pain, GI bleeding, persistent vomiting, jaundice, pregnancy, syncope, or high fever → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Epigastric burning after meals; bloating	Suspected dyspepsia / gastritis
Crampy pain with loose stools after food exposure	Suspected acute gastroenteritis
Colicky right upper quadrant pain after fatty meals	Suspected biliary colic
Lower abdominal pain with urinary symptoms	Suspected lower urinary tract infection

MEDICATION-RELATED DIFFERENTIAL

Recent NSAID use with dyspepsia or epigastric pain → suspected NSAID-related gastritis / dyspepsia.

RELATED DRUG CLASSES

- *Prostanoids, NSAIDs & Leukotriene Antagonists* — NSAID → NSAID-related gastritis/dyspepsia
- *Drugs for Acid-Peptic Diseases* — suspected dyspepsia/gastritis pattern branch

REFERENCES

ACG/CAG dyspepsia guideline, PMID 28631728 | ACG acute abdominal-pain related GI guidance
Drug-induced symptom ref: ACG/CAG Clinical Guideline: Management of Dyspepsia. PMID: 28631728.

7. Diarrhea

SCHOLAR-MAC FOCUS

- Onset, frequency, and stool character: watery, bloody, or mucoid?
- Recent travel, unsafe food, antibiotics, or sick contacts?
- Fever, dehydration, abdominal pain, or vomiting?
- Duration < 14 days or persistent/recurrent?

RED FLAGS / REFER

Severe dehydration, blood in stool, high fever, severe abdominal pain, recent hospitalization/antibiotics, or very young/elderly frail patient → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Acute watery diarrhea after food exposure; self-limited	Suspected acute infectious gastroenteritis
Diarrhea after antibiotics or recent admission	Suspected <i>Clostridioides difficile</i> infection
Recurrent abdominal discomfort relieved by defecation	Suspected irritable bowel syndrome
Bloody diarrhea with systemic symptoms	Suspected invasive bacterial diarrhea / inflammatory bowel disease

MEDICATION-RELATED DIFFERENTIAL

Metformin or recent antibiotic use → suspected medication-related diarrhea.

RELATED DRUG CLASSES

- *Antidiabetic Drugs* — metformin → medication-related diarrhea

⚠️ "Recent antibiotic use" is named but not linked — too many candidate antibiotic-class pages, no single unambiguous match.

REFERENCES

IDSA infectious diarrhea guideline, PMID 29053792 | ACG acute diarrheal infection guideline, PMID 27068718

Drug-induced symptom ref: McCreight LJ, et al. Metformin and the gastrointestinal tract. *Diabetologia*. 2016. PMID: 27987248.

8. Constipation / Hemorrhoids

SCHOLAR-MAC FOCUS

- Infrequent hard stool, straining, incomplete evacuation, or anal pain?
- How long has it been present, and what is the baseline pattern?
- Low fiber/fluid, inactivity, pregnancy, or constipating medicines?
- Rectal bleeding, weight loss, or anemia symptoms?

RED FLAGS / REFER

Severe abdominal distension, vomiting, obstipation, heavy rectal bleeding, weight loss, or new bowel change in older adults → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Hard infrequent stools and straining	Suspected functional constipation
Bright red blood on paper with painful defecation	Suspected anal fissure
Painless bright red bleeding or prolapsing lump with straining	Suspected hemorrhoids
Alternating bowel habits with abdominal discomfort	Suspected irritable bowel syndrome

MEDICATION-RELATED DIFFERENTIAL

Opioids, iron, or anticholinergics → suspected medication-related constipation.

RELATED DRUG CLASSES

- *Analgesics I — Opioids & Paracetamol* — opioids → medication-related constipation
- *Cholinergic Agents — Parasympathomimetics & Parasympatholytics* — anticholinergics → medication-related constipation

⚠️ "Iron" is named but not linked — no drug-class page covers iron supplementation.

REFERENCES

AGA constipation technical review, PMID 23261065 | ASCRS hemorrhoid clinical practice guideline
Drug-induced symptom ref: Crockett SD, et al. AGA guideline on the medical management of opioid-induced constipation. Gastroenterology. 2019. PMID: 30660725.

9. Dysuria / Urinary Symptoms

SCHOLAR-MAC FOCUS

- Burning, frequency, urgency, suprapubic pain, or hematuria?
- Vaginal symptoms or urethral discharge?
- Fever, flank pain, vomiting, pregnancy, or male patient?
- Previous UTI or recent sexual exposure?

RED FLAGS / REFER

Fever, flank pain, vomiting, sepsis features, pregnancy, male UTI, or gross hematuria with clots → same-day medical referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Frequency and dysuria without vaginal symptoms	Suspected acute uncomplicated cystitis
Dysuria with vaginal itch/discharge	Suspected vaginitis rather than cystitis
Flank pain, fever, or systemic illness	Suspected pyelonephritis
Dysuria after sexual exposure with discharge	Suspected urethritis / sexually transmitted infection

MEDICATION-RELATED DIFFERENTIAL

Recent SGLT2 inhibitor use with dysuria/itch → suspected medication-related genital or urinary irritation.

RELATED DRUG CLASSES

- *Antidiabetic Drugs* — SGLT2 inhibitor → medication-related genital/urinary irritation

REFERENCES

IDSA uncomplicated cystitis/pyelonephritis guideline, PMID 21292654 | CDC STI Treatment Guidelines 2021, PMID 34292926
Drug-induced symptom ref: Liu J, et al. Risk of genital and urinary tract infections associated with SGLT2 inhibitors. Sci Rep. 2017. PMID: 28674457.

10. Vaginal Discharge

SCHOLAR-MAC FOCUS

- Color, odor, amount, and duration of discharge?
- Associated itch, burning, dyspareunia, or pelvic pain?
- Pregnancy, fever, bleeding, or STI exposure?
- Recent antibiotic use or diabetes?

RED FLAGS / REFER

Pelvic pain, fever, pregnancy, postmenopausal bleeding, severe vulvar pain, or sexual assault concern → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Thick white discharge with vulvar itch	Suspected vulvovaginal candidiasis
Thin gray discharge with fishy odor	Suspected bacterial vaginosis
Frothy yellow-green discharge with irritation	Suspected trichomoniasis
Pelvic pain or cervical motion tenderness symptoms	Suspected pelvic inflammatory disease

MEDICATION-RELATED DIFFERENTIAL

Antibiotics or SGLT2 inhibitors → suspected medication-related vulvovaginal candidiasis.

RELATED DRUG CLASSES

- *Antidiabetic Drugs* — SGLT2 inhibitor → medication-related vulvovaginal candidiasis
 - *Antifungal Drugs* — suspected vulvovaginal candidiasis pattern branch
- ⚠️ "Antibiotics" is named but not linked — too many candidate antibiotic-class pages, no single unambiguous match.

REFERENCES

CDC STI Treatment Guidelines 2021, PMID 34292926 | ACOG vaginitis patient/clinical resources

Drug-induced symptom ref: Puckrin R, et al. Sodium-glucose cotransporter 2 inhibitors and genital mycotic infection. Diabetes Obes Metab. 2018. PMID: 30136398.

11. Wound / Minor Injury

SCHOLAR-MAC FOCUS

- Abrasion, laceration, puncture, bite, or contusion?
- Time since injury and mechanism?
- Depth, contamination, foreign body, or bleeding?
- Tetanus status, diabetes, or poor wound healing risk?

RED FLAGS / REFER

Deep wound, uncontrolled bleeding, bite wound, foreign body, tendon injury, numbness, or spreading redness/pus → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Superficial clean abrasion or small cut	Suspected minor uncomplicated wound
Tender bruising after blunt trauma	Suspected soft tissue contusion
Redness, warmth, discharge, increasing pain	Suspected infected wound / cellulitis
Puncture or bite wound	Suspected higher-risk wound needing medical review

MEDICATION-RELATED DIFFERENTIAL

Anticoagulants or antiplatelets with excessive bruising/bleeding after minor injury → suspected medication-related bleeding tendency.

RELATED DRUG CLASSES

- *Antithrombotics* — anticoagulants/antiplatelets → medication-related bleeding tendency

REFERENCES

AAFP Laceration Repair: A Practical Approach | CDC wound management to prevent tetanus

Drug-induced symptom ref: ACC Expert Consensus Decision Pathway on Management of Bleeding in Patients on Oral Anticoagulants. J Am Coll Cardiol. 2020. PMID: 32138938.

12. Skin Rash

SCHOLAR-MAC FOCUS

- Macules, papules, vesicles, scales, wheals, or annular lesions?
- Itch, pain, fever, or new product/exposure?
- Localized or generalized?
- Medication history and similar previous episodes?

RED FLAGS / REFER

Non-blanching rash, mucosal involvement, extensive blistering, skin pain, facial swelling, or systemic illness → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Transient itchy wheals	Suspected urticaria
Localized itchy rash after new exposure	Suspected contact dermatitis
Annular scaly plaque with central clearing	Suspected tinea corporis
Greasy scale on scalp/face or flexural itch	Suspected seborrheic dermatitis / eczema

MEDICATION-RELATED DIFFERENTIAL

New antibiotic, NSAID, or anticonvulsant exposure → suspected morbilliform drug eruption.

RELATED DRUG CLASSES

- *Prostanoids, NSAIDs & Leukotriene Antagonists* — NSAID → morbilliform drug eruption
 - *Antiepileptics (Antiseizure Medications)* — anticonvulsant → morbilliform drug eruption
- ⚠ "New antibiotic" is named but not linked — too many candidate antibiotic-class pages, no single unambiguous match.

REFERENCES

AAFP The Generalized Rash: Diagnostic Approach | AAD patient/clinical dermatology resources
Drug-induced symptom ref: AAFP. Adverse Cutaneous Drug Reactions.

13. Eye Symptoms

SCHOLAR-MAC FOCUS

- Redness, itch, discharge, pain, foreign body sensation, or blurred vision?
- One eye or both? Contact lens use?
- Photophobia, trauma, or chemical exposure?
- Lid swelling or localized painful bump?

RED FLAGS / REFER

Reduced vision, severe pain, photophobia, contact lens-related red eye, trauma, or chemical splash
→ urgent ophthalmic referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Itchy watery bilateral eyes	Suspected allergic conjunctivitis
Red gritty eye with discharge and sticky lids	Suspected conjunctivitis
Localized tender lid bump	Suspected hordeolum / sty
Severe painful red eye with blurred vision	Suspected keratitis / uveitis / acute angle-closure glaucoma

MEDICATION-RELATED DIFFERENTIAL

Antihistamines, antidepressants, or anticholinergics → suspected medication-induced dry eye / blurred vision.

RELATED DRUG CLASSES

- *Histamine, Antihistamines & Serotonin Agonists* — antihistamines → medication-induced dry eye/blurred vision
 - *Antidepressants and Lithium* — antidepressants → medication-induced dry eye/blurred vision
- ⚠ "Anticholinergics" is named alongside antihistamines/antidepressants but not linked separately — the antihistamine and antidepressant links above already cover the two anticholinergic-heavy classes named in this deck.

REFERENCES

JAMA conjunctivitis systematic review, PMID 24150468 | American Academy of Ophthalmology: Preferred Practice Patterns
Drug-induced symptom ref: American Academy of Ophthalmology. Common Drugs That Can Worsen Dry Eye.

14. Ear Symptoms

SCHOLAR-MAC FOCUS

- Ear pain, fullness, discharge, itch, or hearing loss?
- Recent swimming, URTI, or ear cleaning?
- Fever, vertigo, or facial weakness?
- One ear or both?

RED FLAGS / REFER

Mastoid swelling, severe systemic illness, facial weakness, severe vertigo, or severe pain in high-risk patients → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Pain worse when pulling the pinna or tragus	Suspected otitis externa
Ear pain/fullness after URTI	Suspected otitis media / eustachian tube dysfunction
Blocked ear and reduced hearing without pain	Suspected cerumen impaction
Ear discharge with hearing loss or chronic symptoms	Suspected tympanic membrane pathology / chronic infection

MEDICATION-RELATED DIFFERENTIAL

Aminoglycosides or loop diuretics with tinnitus/hearing change → suspected drug-induced ototoxicity.

RELATED DRUG CLASSES

- *Diuretics* — loop diuretics → drug-induced ototoxicity
- *Fluoroquinolones, Tetracyclines & Inhibitors of DNA / Protein Synthesis* — aminoglycosides → drug-induced ototoxicity (dedicated Aminoglycosides section)

REFERENCES

AAO-HNS otitis externa guideline, PMID 24491310 | AAP acute otitis media guideline, PMID 23439909

Drug-induced symptom ref: Schacht J, et al. Ototoxicity: aminoglycosides and cisplatin. Cold Spring Harb Perspect Med. 2012. PMID: 23028175.

15. Fever, Cough, and/or Sore Throat

SCHOLAR-MAC FOCUS

- Main complaint: fever, cough, sore throat, hoarseness, or rhinorrhea?
- Duration, exposure, and severity?
- Dyspnea, chest pain, drooling, or inability to swallow?
- Comorbidities, age extremes, or immunosuppression?

RED FLAGS / REFER

Respiratory distress, hypoxia, chest pain, drooling, stridor, sepsis features, or dehydration → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Runny nose + cough + sore throat; self-limited	Suspected viral upper respiratory infection
Sore throat with fever and tonsillar exudate; no cough	Suspected streptococcal pharyngitis
Cough after viral illness without dyspnea	Suspected acute bronchitis
Fever with pleuritic chest pain or dyspnea	Suspected pneumonia

MEDICATION-RELATED DIFFERENTIAL

ACE inhibitor use → suspected ACE inhibitor-related cough.

RELATED DRUG CLASSES

- *ACE Inhibitors, ARBs & DRIs* — ACE inhibitor → ACE inhibitor-related cough

REFERENCES

ACP/CDC acute respiratory tract infection guideline, PMID 26785402 | IDSA Group A streptococcal pharyngitis guideline, PMID 22965026
Drug-induced symptom ref: Dicipinigitis PV. Angiotensin-converting enzyme inhibitor-induced cough. Chest. 2006. PMID: 16428706.

16. COVID-19-compatible Illness

SCHOLAR-MAC FOCUS

- Fever, cough, sore throat, myalgia, anosmia, or exposure history?
- How many days since symptom onset?
- Dyspnea, chest pain, confusion, or reduced oral intake?
- Age and high-risk comorbidities?

RED FLAGS / REFER

Dyspnea, hypoxia, chest pain, confusion, cyanosis, or inability to maintain hydration → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Acute URI symptoms with exposure or compatible syndrome	Suspected COVID-19
Fever and myalgia during community outbreaks	Suspected viral respiratory infection / influenza-like illness
Predominant nasal allergy symptoms without fever	Suspected allergic rhinitis rather than COVID-19
Worsening respiratory symptoms after several days	Suspected lower respiratory tract involvement

MEDICATION-RELATED DIFFERENTIAL

Recent antibiotic, anticonvulsant, or allopurinol exposure with fever → suspected drug fever.

RELATED DRUG CLASSES

- *Antiepileptics (Antiseizure Medications)* — anticonvulsant → drug fever
- *Drugs for Gout* — allopurinol → drug fever

⚠ "Recent antibiotic" is named but not linked — too many candidate antibiotic-class pages, no single unambiguous match.

REFERENCES

NIH COVID-19 Treatment Guidelines | IDSA COVID-19 treatment and management guideline

Drug-induced symptom ref: Patel RA, Gallagher JC. Drug fever. *Pharmacotherapy*. 2010. PMID: 20180602.

17. Runny Nose / Nasal Congestion

SCHOLAR-MAC FOCUS

- Watery or purulent discharge? Sneezing and itchy nose?
- Duration < 10 days or longer?
- Facial pain, fever, or reduced smell?
- One-sided discharge or child with foreign body concern?

RED FLAGS / REFER

Orbital swelling, severe headache, high fever, facial swelling, or prolonged unilateral foul-smelling discharge → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Sneezing, itchy eyes, clear rhinorrhea	Suspected allergic rhinitis
Acute congestion with sore throat/cough	Suspected viral rhinitis / common cold
Facial pressure and purulent discharge >10 days or double worsening	Suspected acute bacterial rhinosinusitis
One-sided foul discharge in a child	Suspected nasal foreign body

MEDICATION-RELATED DIFFERENTIAL

Frequent topical nasal decongestant use → suspected rhinitis medicamentosa.

RELATED DRUG CLASSES

- *Adrenergic Drugs — Sympathomimetics & Sympatholytics* — topical nasal decongestant (adrenergic agonist) → rhinitis medicamentosa

REFERENCES

AAO-HNS allergic rhinitis guideline, PMID 25644617 | AAO-HNS adult sinusitis guideline, PMID 25832968
Drug-induced symptom ref: AAFP. Rhinitis Medicamentosa.

18. Mouth Ulcer

SCHOLAR-MAC FOCUS

- Single or multiple ulcers? Painful or painless?
- Duration and recurrence?
- Trauma from teeth/denture or hot food?
- Fever, weight loss, or immunosuppression?

RED FLAGS / REFER

Ulcer lasting > 2–3 weeks, induration, unexplained weight loss, severe dehydration, or immunocompromised host → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Small painful shallow recurrent ulcers	Suspected aphthous ulcer
Ulcer after cheek bite, brace, or denture trauma	Suspected traumatic ulcer
White plaques or soreness with steroid use	Suspected oral candidiasis
Persistent non-healing ulcer	Suspected oral malignancy until evaluated

MEDICATION-RELATED DIFFERENTIAL

Methotrexate exposure with painful oral ulcers → suspected medication-induced mucositis / oral ulceration.

RELATED DRUG CLASSES

None linked — see note below.

⚠️ "Methotrexate" is named but not linked — it appears in two of the 52 pages (drug-class-ibd, drug-class-anticancer-1) for two different indications, and the deck doesn't say which use is meant; picking one would be guessing.

REFERENCES

Recurrent aphthous stomatitis review, PMID 24210299 | AAFP Common Oral Lesions

Drug-induced symptom ref: Kalantzis A, et al. Oral ulceration due to methotrexate. J Oral Pathol Med. 2005. PMID: 15752195.

19. Cold Sore / Lip Vesicle

SCHOLAR-MAC FOCUS

- Tingling followed by grouped vesicles on the lip?
- Recurrent or first episode?
- Triggers such as fever, stress, or sun exposure?
- Eye symptoms or widespread lesions?

RED FLAGS / REFER

Eye involvement, immunocompromised patient, severe primary infection, or inability to drink/eat → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Recurrent grouped vesicles on lip border	Suspected herpes labialis
Cracking at mouth corners	Suspected angular cheilitis
Diffuse painful oral lesions with fever	Suspected primary herpetic gingivostomatitis
Honey-colored crusted lesion around mouth	Suspected impetigo

MEDICATION-RELATED DIFFERENTIAL

NSAIDs, tetracyclines, or TMP-SMX with recurrent lip lesion → suspected fixed drug eruption.

RELATED DRUG CLASSES

- *Prostanoids, NSAIDs & Leukotriene Antagonists* — NSAID → fixed drug eruption
- *Fluoroquinolones, Tetracyclines & Inhibitors of DNA / Protein Synthesis* — tetracycline and TMP-SMX → fixed drug eruption (covers both, dedicated sections)

REFERENCES

HSV-1 oral infection review, PMID 18154571 | AAFP Nongenital Herpes Simplex Virus

Drug-induced symptom ref: AAFP. Adverse Cutaneous Drug Reactions (fixed drug eruption is a classic presentation).

20. Minor Burn / Scald

SCHOLAR-MAC FOCUS

- Cause: hot liquid, contact, steam, or sun?
- Size, depth, and body location?
- Time since injury and first aid already used?
- Pain only, or blisters/charring?

RED FLAGS / REFER

Burn to face, hands, feet, genitalia, major joints, large area, deep burn, chemical/electrical burn, or inhalation injury → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Red painful skin without blistering	Suspected superficial burn
Small blistering burn with intact circulation	Suspected superficial partial-thickness burn
Pale, leathery, painless, or circumferential burn	Suspected deep burn
Sun exposure with diffuse erythema	Suspected sunburn

MEDICATION-RELATED DIFFERENTIAL

Recent doxycycline, thiazide, or amiodarone use with sun-exposed burn-like rash → suspected phototoxic drug reaction.

RELATED DRUG CLASSES

- *Fluoroquinolones, Tetracyclines & Inhibitors of DNA / Protein Synthesis* — doxycycline → phototoxic drug reaction
- *Diuretics* — thiazide → phototoxic drug reaction
- *Antiarrhythmic Drugs* — amiodarone → phototoxic drug reaction

REFERENCES

American Burn Association referral guidelines | AAFP Outpatient Burn Care
Drug-induced symptom ref: Drucker AM, Rosen CF. Drug-induced photosensitivity. Drug Saf. 2011. PMID: 21288010.

21. Itching of Skin / Scalp

SCHOLAR-MAC FOCUS

- Localized or generalized itch? Dry skin, flakes, rash, or bites?
- Worse at night or after new product exposure?
- Scalp scaling, dandruff, or nits?
- Jaundice, weight loss, or systemic symptoms?

RED FLAGS / REFER

Generalized unexplained itch with jaundice/systemic symptoms, severe skin infection, or extensive excoriation → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Dry itchy skin without primary rash	Suspected xerosis / irritant itch
Scalp flaking and itch	Suspected seborrheic dermatitis / dandruff
Itchy flexural or allergic-type rash	Suspected eczema / allergic dermatitis
Night itch with household contacts affected	Suspected scabies

MEDICATION-RELATED DIFFERENTIAL

Opioids or new medications causing generalized itch → suspected drug-induced pruritus.

RELATED DRUG CLASSES

- *Analgesics I — Opioids & Paracetamol* — opioids → drug-induced pruritus

REFERENCES

AAFP Pruritus: Diagnosis and Management | AAD Atopic dermatitis clinical guideline
Drug-induced symptom ref: AAFP. Pruritus: Diagnosis and Management.

22. Worm Infection / Helminthic Symptoms

SCHOLAR-MAC FOCUS

- Perianal itch, visible worms, abdominal discomfort, or weight loss?
- Night-time anal itching in children?
- History of barefoot exposure or unsafe food/water?
- Anemia symptoms or severe abdominal pain?

RED FLAGS / REFER

Severe abdominal pain, bowel obstruction symptoms, significant anemia, or malnutrition → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Night-time perianal itch, especially in children	Suspected enterobiasis / pinworm infection
Passage of roundworms or abdominal discomfort	Suspected ascariasis
Iron deficiency symptoms with soil exposure	Suspected hookworm infection
Chronic abdominal complaints after unsafe food/water	Suspected intestinal helminth infection

MEDICATION-RELATED DIFFERENTIAL

Laxative overuse, suppositories, or antibiotic-associated candidiasis → suspected medication-related perianal irritation mimicking worms.

RELATED DRUG CLASSES

- *Antiparasitic Drugs* — the suspected helminth infections above (pinworm/ascariasis/hookworm) — treatment class
- ⚠ "Laxative" and "suppositories" are named but not linked — no drug-class page covers plain laxatives.

REFERENCES

CDC pinworm/enterobiasis clinical overview | Soil-transmitted helminths review, PMID 28882382
Drug-induced symptom ref: AAFP. Pruritus Ani and common anorectal symptom differentials.

23. Scabies / Lice

SCHOLAR-MAC FOCUS

- Severe itch, worse at night, and affected contacts?
- Burrows on finger webs, wrists, waist, or genital area?
- Scalp itch with visible nits or lice?
- Household or school outbreak?

RED FLAGS / REFER

Crusted scabies, secondary bacterial infection, infant/frail elderly, or extensive involvement → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Nocturnal itch with burrows and household spread	Suspected scabies
Scalp itch with nits attached to hair shafts	Suspected pediculosis capitis
Pubic itch with nits in coarse hair	Suspected pediculosis pubis
Excoriated itchy lesions with pustules	Suspected infestation with secondary infection

MEDICATION-RELATED DIFFERENTIAL

New antibiotic, NSAID, or opioid exposure with itchy papules → suspected drug eruption mimicking infestation.

RELATED DRUG CLASSES

None linked — see note below.

⚠️ No link made — the deck's suspected diagnoses here (scabies, pediculosis) are treated with topical scabicides/pediculicides (e.g. permethrin), and none of the 52 drug-class pages covers that as a class (drug-class-antiparasitic.md is helminth-focused only, confirmed by content check, not ectoparasites). This is a real content gap, not an extraction shortcut — worth flagging if the drug-class set grows.

REFERENCES

CDC STI Treatment Guidelines: ectoparasitic infections, PMID 34292926 | AAFP Pediculosis and Scabies
Drug-induced symptom ref: AAFP. Adverse Cutaneous Drug Reactions.

24. Abscess / Skin Boil

SCHOLAR-MAC FOCUS

- Painful lump, fluctuation, pus, or surrounding redness?
- Site: skin, axilla, groin, buttock?
- Fever, rapid enlargement, or recurrent lesions?
- Diabetes or immunocompromised state?

RED FLAGS / REFER

Large abscess, facial/periorbital abscess, spreading cellulitis, fever, or immunocompromised patient → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Tender fluctuant nodule with central pus point	Suspected skin abscess / furuncle
Multiple recurrent nodules in axilla/groin	Suspected hidradenitis suppurativa
Diffuse erythema and warmth without fluctuation	Suspected cellulitis
Painful red lesion around hair follicle	Suspected folliculitis / furunculosis

MEDICATION-RELATED DIFFERENTIAL

Systemic corticosteroid use → suspected steroid-associated folliculitis / acneiform eruption.

RELATED DRUG CLASSES

- *Adrenal Steroids & Their Antagonists* — systemic corticosteroid → steroid-associated folliculitis/acneiform eruption

REFERENCES

IDSA skin and soft tissue infection guideline, PMID 24947530 | IDSA SSTI official guideline

Drug-induced symptom ref: DermNet NZ / acneiform eruptions are classically caused by corticosteroids; corroborated in dermatology reviews. PMID: 32759293.

25. Numbness / Paresthesia

SCHOLAR-MAC FOCUS

- Sudden or gradual? One limb, one side, or symmetrical distal numbness?
- Weakness, gait change, speech difficulty, or bladder symptoms?
- Back/neck pain or repetitive posture?
- History of diabetes, hyperventilation, or anxiety?

RED FLAGS / REFER

Sudden focal deficit, facial droop, severe weakness, saddle anesthesia, or bladder/bowel dysfunction → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Brief numbness after pressure on a limb	Suspected transient nerve compression
Numb thumb/index/middle fingers, worse at night	Suspected carpal tunnel syndrome
Burning numb feet in diabetes	Suspected peripheral neuropathy
Sudden unilateral numbness with weakness or speech change	Suspected stroke / transient ischemic attack

MEDICATION-RELATED DIFFERENTIAL

Metronidazole or isoniazid exposure with distal tingling/numbness → suspected medication-induced peripheral neuropathy.

RELATED DRUG CLASSES

- *Antimycobacterial Drugs* — isoniazid → medication-induced peripheral neuropathy
- *Fluoroquinolones, Tetracyclines & Inhibitors of DNA / Protein Synthesis* — metronidazole → medication-induced peripheral neuropathy (dedicated Metronidazole section)
- *Analgesics II — Neuropathic Pain & Centrally Acting Muscle Relaxants* — suspected peripheral neuropathy pattern branch — treatment class

REFERENCES

AHA/ASA acute ischemic stroke guideline, PMID 31662037 | AAFP Peripheral Neuropathy

Drug-induced symptom ref: AAFP. Peripheral Neuropathy: Evaluation and Differential Diagnosis (lists medication causes).

26. Insomnia

SCHOLAR-MAC FOCUS

- Difficulty falling asleep, staying asleep, or early waking?
- How long has it been present and what is the sleep schedule?
- Stress, caffeine, alcohol, medications, or screen use?
- Depression, anxiety, snoring, or daytime sleepiness?

RED FLAGS / REFER

Suicidal ideation, mania, psychosis, substance withdrawal, or severe daytime impairment → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Short-term insomnia during stress	Suspected adjustment / acute insomnia
Irregular sleep schedule and poor sleep habits	Suspected behavioral insomnia / poor sleep hygiene
Loud snoring and daytime sleepiness	Suspected obstructive sleep apnea
Low mood, anhedonia, or anxiety symptoms	Suspected mood or anxiety disorder-related insomnia

MEDICATION-RELATED DIFFERENTIAL

Corticosteroids, decongestants, stimulants, or excess caffeine → suspected medication-related insomnia.

RELATED DRUG CLASSES

- *Adrenal Steroids & Their Antagonists* — corticosteroids → medication-related insomnia
 - *CNS Stimulants and Drugs Affecting Appetite* — stimulants → medication-related insomnia
 - *Anxiolytics and Hypnotics* — pharmacologic treatment class for insomnia
- ⚠ "Decongestants" is named but not linked separately — see [17. Runny Nose / Nasal Congestion](#) for that link (drug-class-adrenergic), not duplicated here.

REFERENCES

AASM insomnia pharmacologic guideline, PMID 27998379 | ACP chronic insomnia CBT-I guideline, PMID 27136449

Drug-induced symptom ref: AAFP. Insomnia: Pharmacologic Therapy; evaluate medication causes and sleep-hygiene contributors.

27. Motion Sickness

SCHOLAR-MAC FOCUS

- Does nausea/dizziness occur specifically during travel or motion exposure?
- History of migraine or previous motion sickness?
- Symptoms only with movement or also at rest?
- Vomiting severe enough to impair hydration?

RED FLAGS / REFER

Persistent vertigo at rest, severe headache, neurologic deficit, or repeated vomiting with dehydration → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Nausea and dizziness only during travel	Suspected motion sickness
Motion-triggered headache with photophobia	Suspected migraine-associated motion sensitivity
Vertigo even without travel exposure	Suspected vestibular disorder rather than simple motion sickness
Significant anticipatory nausea	Suspected conditioned / anxiety-related motion sensitivity

MEDICATION-RELATED DIFFERENTIAL

Opioids or SSRIs causing nausea/dizziness during travel → suspected medication-induced symptoms mimicking motion sickness.

RELATED DRUG CLASSES

- *Analgesics / — Opioids & Paracetamol* — opioids → medication-induced symptoms mimicking motion sickness
- *Antidepressants and Lithium* — SSRIs → medication-induced symptoms mimicking motion sickness

REFERENCES

Motion sickness susceptibility review, PMID 16952441 | CDC Yellow Book: motion sickness

Drug-induced symptom ref: CDC Yellow Book: motion sickness differential includes other causes of nausea/dizziness.

28. Loss of Appetite / Anorexia

SCHOLAR-MAC FOCUS

- Acute or chronic loss of appetite?
- Associated nausea, abdominal pain, weight loss, or fever?
- Stress, grief, depression, or medication change?
- Dysphagia or early satiety?

RED FLAGS / REFER

Significant weight loss, dehydration, persistent vomiting, dysphagia, severe depression, or unexplained fever → referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Short-term reduced appetite during stress or minor illness	Suspected functional / stress-related anorexia
Epigastric discomfort, nausea, bloating	Suspected dyspepsia / gastritis
Low mood, sleep change, anhedonia	Suspected depressive disorder
Progressive weight loss or early satiety	Suspected underlying systemic or gastrointestinal disease

MEDICATION-RELATED DIFFERENTIAL

GLP-1 receptor agonist use → suspected medication-related loss of appetite.

RELATED DRUG CLASSES

- *Antidiabetic Drugs* — GLP-1 receptor agonist → medication-related loss of appetite

REFERENCES

Functional dyspepsia review, PMID 26535514 | AAFP Unintentional Weight Loss in Older Adults
Drug-induced symptom ref: AAFP. Semaglutide (Wegovy) for the Treatment of Obesity.

29. Nausea / Vomiting

SCHOLAR-MAC FOCUS

- Onset, frequency, and triggers?
- Associated abdominal pain, diarrhea, fever, headache, or pregnancy?
- Oral intake and hydration status?
- Blood, bile, or severe persistent vomiting?

RED FLAGS / REFER

Severe dehydration, bilious or bloody vomit, severe abdominal pain, altered mental status, or pregnancy complications → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Acute nausea with diarrhea after food exposure	Suspected gastroenteritis / food poisoning
Morning nausea with missed period	Suspected pregnancy-related nausea
Nausea with epigastric discomfort	Suspected gastritis / dyspepsia
Nausea with headache/photophobia	Suspected migraine-associated nausea

MEDICATION-RELATED DIFFERENTIAL

GLP-1 receptor agonists, metformin, or opioids → suspected medication-related nausea / vomiting.

RELATED DRUG CLASSES

- *Antidiabetic Drugs* — GLP-1 receptor agonist/metformin → medication-related nausea/vomiting
- *Analgesics I — Opioids & Paracetamol* — opioids → medication-related nausea/vomiting

REFERENCES

AAFP Evaluation of Nausea and Vomiting | ACOG Nausea and Vomiting of Pregnancy

Drug-induced symptom ref: Kushner RF, et al. Semaglutide 2.4 mg for obesity: GI adverse effects common. N Engl J Med. 2021. PMID: 33567185.

30. Minor Allergy / Food Allergy / Insect Bite

SCHOLAR-MAC FOCUS

- Trigger: food, medicine, environment, or insect bite?
- Localized itch vs generalized hives?
- Lip/tongue swelling, wheeze, or dizziness?
- Previous similar reactions?

RED FLAGS / REFER

Breathing difficulty, hypotension, syncope, angioedema, or rapidly progressive generalized reaction → emergency referral for suspected anaphylaxis.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Localized itchy wheal at bite site	Suspected local insect-bite reaction
Sneezing, itchy eyes, runny nose after exposure	Suspected allergic rhinitis / allergic conjunctivitis
Generalized itchy wheals without instability	Suspected acute urticaria
Lip/tongue swelling or systemic symptoms after food/drug	Suspected systemic allergic reaction / anaphylaxis

MEDICATION-RELATED DIFFERENTIAL

Penicillins, sulfonamides, or NSAIDs → suspected drug-induced urticaria / allergic reaction.

RELATED DRUG CLASSES

- *Drugs Affecting Cell Wall Synthesis (β -Lactams)* — penicillins → drug-induced urticaria/allergic reaction
- *Fluoroquinolones, Tetracyclines & Inhibitors of DNA / Protein Synthesis* — sulfonamides (TMP-SMX, Folate Synthesis Inhibitors section) → drug-induced urticaria/allergic reaction
- *Prostanoids, NSAIDs & Leukotriene Antagonists* — NSAIDs → drug-induced urticaria/allergic reaction
- *Histamine, Antihistamines & Serotonin Agonists* — treatment class for acute urticaria/allergic reaction

REFERENCES

AAAAI/ACAAI anaphylaxis practice parameter, PMID 32001253 | AAAAI/ACAAI urticaria practice parameter, PMID 25023742
Drug-induced symptom ref: Joint Task Force Practice Parameter. Anaphylaxis 2020 update. PMID: 32001253.

31. Smoking-related Illness / Symptoms in a Smoker

SCHOLAR-MAC FOCUS

- Cough, sputum, wheeze, dyspnea, chest pain, or hemoptysis?
- Acute worsening or chronic baseline?
- Smoking intensity and duration?
- Weight loss, fever, or cardiovascular symptoms?

RED FLAGS / REFER

Chest pain, hemoptysis, hypoxia, severe dyspnea, syncope, or unexplained weight loss → urgent referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Chronic productive cough in a smoker	Suspected chronic bronchitis / COPD-related symptom
Cough after viral illness without hypoxia	Suspected acute bronchitis in a smoker
Exertional dyspnea and wheeze	Suspected obstructive airway disease
Hemoptysis or weight loss	Suspected serious smoking-related disease, including lung cancer

MEDICATION-RELATED DIFFERENTIAL

ACE inhibitors or nonselective beta-blockers with cough/wheeze → suspected medication-related respiratory symptoms.

RELATED DRUG CLASSES

- *ACE Inhibitors, ARBs & DRLs* — ACE inhibitor → medication-related respiratory symptoms

⚠️ "Nonselective beta-blockers" is named but not linked — beta-blockers are covered across four different pages (antihypertensives, antianginal, antiarrhythmic, heart-failure) with no single page as the unambiguous home; picking one would be guessing.

REFERENCES

GOLD COPD report | USPSTF tobacco smoking cessation, PMID 33464343

Drug-induced symptom ref: Dicipinigitis PV. ACE inhibitor-induced cough. Chest. 2006. PMID: 16428706.

32. Gingivitis / Halitosis

SCHOLAR-MAC FOCUS

- Bleeding gums, swollen gums, bad breath, tooth pain, or oral dryness?
- Oral hygiene routine and dental plaque?
- Ulcers, fever, or facial swelling?
- Dry mouth, smoking, or denture use?

RED FLAGS / REFER

Facial swelling, fever, trismus, severe pain, or persistent oral lesion > 2–3 weeks → dental/medical referral.

If uncertain, use safety-netting and arrange follow-up (see [CPT1 common-illness — shared red flags & patient context](#) for the shared referral table and patient-context checklist).

Any red flag, severe illness, or high-risk patient?

YES → same-day / urgent referral. Use emergency referral when unstable.

NO → identify the dominant illness-script pattern below.

ILLNESS-SCRIPT PATTERNS

PRESENTATION	SUSPECTED DIAGNOSIS
Bleeding gums with plaque and mild tenderness	Suspected gingivitis
Loose teeth, gum recession, chronic inflammation	Suspected periodontitis
Bad breath with dry mouth or tongue coating	Suspected halitosis related to poor oral hygiene / xerostomia
Fever with severe gum pain and necrotic papillae	Suspected acute necrotizing gingival infection

MEDICATION-RELATED DIFFERENTIAL

Phenytoin, amlodipine, or cyclosporine → suspected drug-induced gingival enlargement; anticholinergics may worsen halitosis via xerostomia.

RELATED DRUG CLASSES

- *Antiepileptics (Antiseizure Medications)* — phenytoin → drug-induced gingival enlargement
- *Calcium Channel Blockers* — amlodipine → drug-induced gingival enlargement
- *Immunomodulators — Immunosuppressors & Immunostimulators* — cyclosporine → drug-induced gingival enlargement

REFERENCES

American Academy of Periodontology resources | AAFP Common Dental Emergencies

Drug-induced symptom ref: American Academy of Periodontology. Drug-influenced gingival enlargement resources.